

REPRODUCTIVE HEALTH SUB-WORKING GROUP RHSWG

NEWSLETTER ISSUE #7

JUL-SEP 2025

In Lebanon, the Reproductive Health Sub-Working Group (RHSWG) was established in 2013 under the Health Sector. The Ministry of Public Health (MOPH) leads the RHSWG where the United Nations Population Fund (UNFPA) serves as the co-lead. The RHSWG members include national and international non-governmental organizations (NGOs), academics, relevant ministries, and UN agencies who coordinate to standardize and improve all aspects of sexual and reproductive health (SRH) programming such as community awareness, service quality, accessibility, and coverage. The RHSWG develops and follows up on the implementation of a yearly action plan that contributes to the objectives laid out in the Lebanese Response Plan (LRP).

This issue highlights priority SRH needs identified and issues faced between July and September 2025

✓ RHSWG Key Achievements Jul-Sep 2025 include:



Maternal and Neonatal Secondary Healthcare

Prioritization Tool for Hospitalization Support for High Risk Pregnancies

- With the aim of prioritizing reproductive health support under the Lebanon Response Plan (LRP) for 2026, the RHSWG has successfully developed and piloted in Sep-Oct the **"Prioritization Tool for Hospitalization Support for High Risk Pregnancies"**
- The tool offers a standard approach for health partners subsidizing deliveries to prioritize birthing support by identifying the most vulnerable women and will be implemented as of Jan 1, 2026
- The health sector will optimize the socio-economic criteria to align with other sectors to ensure interoperability and expand its use to cover additional case mix

Operational Monitoring of "out-of-Hospital" Deliveries

- **The risk for "out-of-hospital" deliveries is projected to increase** with UNHCR's withdrawal of its support to maternal and neonatal services at hospitals as of Dec 1, 2025.
- This type of practice increases the risk of maternal and neonatal mortalities for vulnerable women who are unable to afford the healthcare costs associated with delivery.
- The RHSWG deployed the **"Field Assessment KOBO Questionnaire"** in Sep 2025 for use by outreach health workers to improve the operational monitoring of 'out-of-hospital' deliveries and enhance health monitoring and service planning for mothers and newborns in Lebanon. To ensure sustainability, the RHSWG is working with MOPH on integrating the questionnaire into the MOPH mobile health application (MERA).

Community Midwifery Intervention Workshop

- In July 2025, the RHSWG facilitated a workshop to discuss how to **standardize existing community midwifery interventions and empower midwives** to deliver essential SRH services to vulnerable women at the community level.
- The workshop shed light on ongoing community midwifery interventions following a stock taking exercise by the RHSWG.
- Major **recommendations** for community midwifery interventions included: standardizing services, payment, and expanding implementation of health information systems.
- Requirements for **advancing midwife-led birthing** was recommended as a possible solution for decreasing the cost of institutional delivery.



Dashboard Updates

In an effort to improve referrals between health partners, the RHSWG continues to update its service provision referral matrices for SRH services including: Delivery, NICU, and Clinical Management of Rape (CMR) services at supported hospitals and CMR facilities.

- ➔ [Hospital_Delivery Support Dashboard_Nov6_2025](#)
- ➔ [Hospital_NICU Support Dashboard_Nov6_2025](#)
- ➔ [Clinical Management of Rape \(CMR\) Dashboard_Sep24_2025](#)



EVENTS

Launching The Dietary Guide for Pregnant and Lactating Women

The Ministry of Public Health (MOPH), in partnership with UNFPA and in collaboration with the Lebanese Order of Dietitians and the Lebanon Nutrition Sector launched the Dietary Guide for Pregnant and Lactating Women on October 3, 2025.

**These practical guidelines:**

- ✓ Promote healthy, diverse diets
- ✓ Support women through pregnancy & breastfeeding
- ✓ Serve as a tool for both the public & healthcare providers



"We must remember that the fight for better health and dignity is fought one day at a time, one life at a time, and one meal at a time" - Anandita Philipose, UNFPA Lebanon Representative

This reference guide provides dietary guidelines and recommendations for women during the preconception, pregnancy, lactation, and postpartum periods. It is designed as a practical tool for healthcare professionals; including nurses, midwives, physicians, and dietitians; to support women across these stages. The guide also includes special considerations for women from vulnerable groups, such as those living in low-income households, migrant women, and women with disabilities or mental health conditions.



The Guideline and the relevant Information Education and Communication (IEC) Materials are available [here](#).



CAMPAIGNS

Ministry of Public Health (MOPH)

Breast Cancer Awareness Campaign 2025: Free Mammograms Across Lebanon

Breast cancer remains one of the most prevalent cancers among women worldwide, and despite its seriousness, early detection through timely and regular screening continues to be the most powerful tool for improving survival and treatment success. Recognizing this, **the Ministry of Public Health (MoPH)** has launched its annual **Breast Cancer Awareness Campaign**, offering free mammograms at a network of selected hospitals across Lebanon until the end of the year. This nationwide initiative aims to motivate women to prioritize early screening, as identifying abnormalities at an early stage greatly increases the chances of effective treatment and reduces the risk of complications.

Primary Healthcare Centers (PHCCs) are playing an essential role in strengthening the campaign's reach. Across all regions, PHCC teams are actively referring eligible women to participating hospitals, conducting **awareness sessions**, and engaging with communities to explain the **importance of regular breast exams**. Through both educational efforts and hands-on support, these centers are ensuring that women—especially those in underserved areas—have access to accurate information and essential preventive services.

By **covering the cost of mammograms and removing financial barriers** that often delay early diagnosis, the **MoPH** is reaffirming its commitment to women's health and emphasizing the **importance of preventive care** within Lebanon's healthcare system. The campaign not only promotes early detection but also empowers women to take charge of their health, ensuring that no one is deprived of life-saving screening due to economic constraints.

Scan the QR code
to view the full list
of hospitals offering
free mammograms



الحملة الوطنية للتوعية
من سرطان الثدي

الجمهورية اللبنانية
وزارة الصحة العامة

صرتي بالأربعين
ما تفكري مرتين
عملي صورة الثديين

للاستفسار عن مراكز
الصورة الشعاعية
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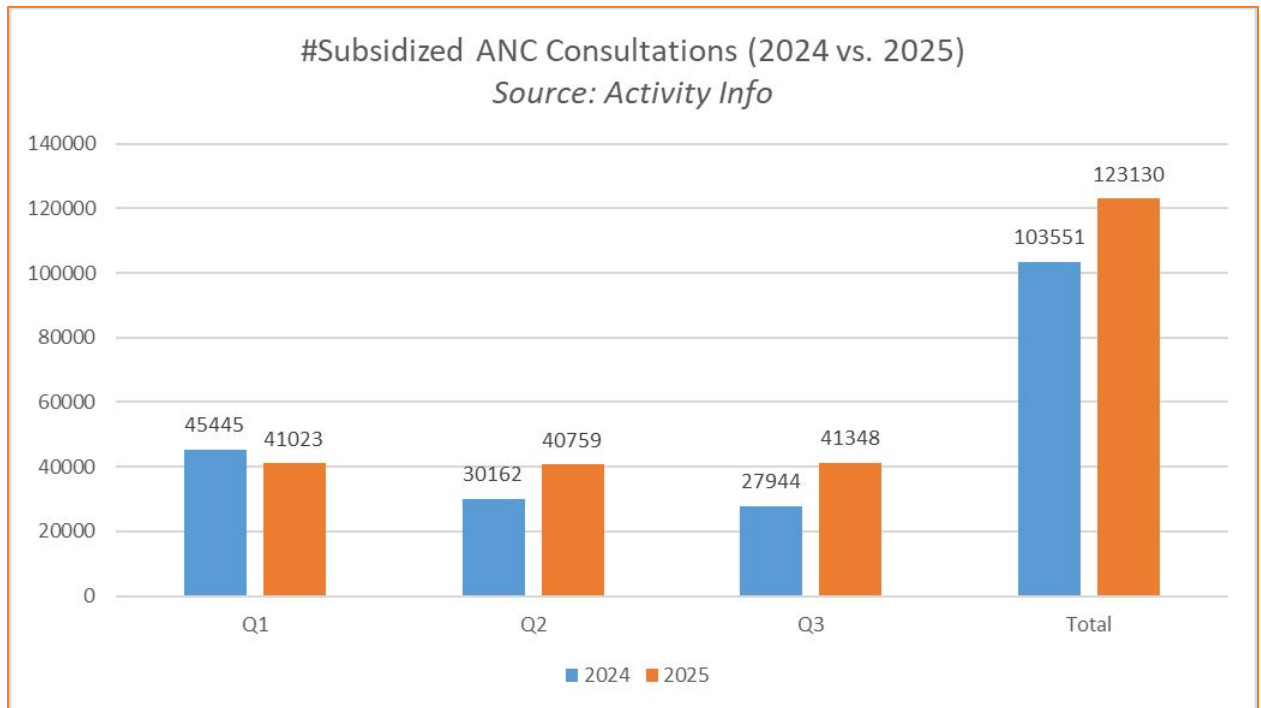
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Primary Healthcare – Service Utilization Trends

The health sector in Lebanon continues to deliver equitable quality healthcare services targeting vulnerable Lebanese and non-Lebanese individuals including displaced Syrians, Palestinian Refugees from Syria (PRS), Palestine Refugees in Lebanon (PRL), migrants, and displaced individuals of diverse nationalities. The health sector supports improved access to comprehensive primary healthcare which includes sexual and reproductive healthcare among other essential health and nutrition services.



Analysis

As of Q3 2025, there is a **19% increase** in the total number of subsidized **Antenatal Care (ANC)** consultations provided in 2025 at **Primary Healthcare Centers (PHCCs)** compared with 2024. This reflects ongoing health partner efforts to enhance access to maternal healthcare at supported PHCCs which helps contribute to decreased maternal and neonatal mortality.

Challenges

Impact of decreased funding and hostilities:

The health sector is still recovering from the devastation inflicted by the hostilities with **13 PHCCs** reported as either fully damaged or closed due to damage. **11** of these are in the Nabatieh Governorate (mainly Bent Jbeil and Marjeyoun Districts), with the remaining **2 PHCCs** in Tyre and Baabda Districts.

Due to funding cuts, there is reduced access to essential sexual and reproductive health services in **18 PHCCs** (health sector, July 2025). Vulnerable groups, including low-income Lebanese families, refugees, and migrants, are unable to pay out-of-pocket.

Reduced access to maternal care (antenatal and postnatal) throughout pregnancy increases the risk of maternal morbidity and mortality. The UNFPA estimates that over the course of 1 year, **8,500 vulnerable pregnant women** will have reduced access to essential maternal health services (**40% of those affected are due to the 13 damaged PHCCs while 60% are due to defunding at 18 PHCCs**). Furthermore, UNFPA estimates that **due to defunding at 18 PHCCs, 14,000 women** will have reduced access to essential SRH services, including family planning.

Post-December 2024 Arrivals from Syria:

Post-2024 arrivals who fled following the change of the regime are mostly residing in Akkar and Baalbak El Hermel. Those present at informal collective shelters live with poor WaSH conditions which put women and girls at increased risk of UTIs and reproductive health tract infections.

Health partners serving these extremely vulnerable populations report barriers to access that include legal status preventing them from seeking services at hospitals located beyond **Lebanese Armed Forces (LAF)** checkpoints, transportation costs, and lack of awareness of available services (most importantly subsidized deliveries at hospitals).

Some projects serving post-2024 arrivals primarily through primary health satellite units (PSUs) are coming to an end in **Q4 2025**. Further support is needed to fill the gaps of defunding and expand SRH service provision to aid the health sector recovery following the hostilities and respond to the SRH needs of Syrian newcomers.



Access to Maternal Care at the Secondary Level

In **Q3 2025**, health partners continued to subsidize deliveries at **35 supported hospitals**.

New projects announced by health partners in September include: Caritas covering high risk pregnancies across Lebanon under LHF funding; Caritas covering all pregnancies in Nabatieh and South under **UNFPA/KOICKA funding**; PUI's project focusing on adolescent pregnancies in Beirut and Baabda under SIDA funding; Mdm extending their support to end of December 2025 for **low risk deliveries** through the midwife-led birthing center and high risk pregnancy support at RHUH, and ACF's extension of its coverage of **high-risk pregnancies** in Bekaa and Baalbak-El Hermel to end of December 2025.

Even though several health partners have stepped in to cover more deliveries, mainly high-risk pregnancies, these projects have low targets, cover all populations, and many close at the end of 2025. While some health partners (ACF, Caritas, and PUI) have secured funding under LHF and ECHO for 2026 to cover high risk pregnancies. **The current and secured interventions are estimated to cover only 40% of the estimated refugee high risk pregnancies in need of support for institutional deliveries as of Dec 1, 2025. Also, commitment towards covering low to medium-risk pregnancies for vulnerable populations and specifically refugees remains lacking.**

As of December 1st, **refugees will no longer be covered by UNHCR** including acute emergency, delivery, and NICU/PICU conditions. Without UNHCR support, the health sector estimates approx. **16,500 vulnerable women and 1000 neonates** will face financial barriers to accessing safe hospital deliveries & NICU services increasing the risk of both maternal and neonatal mortality.

Further support is needed as the health sector continues to advocate with donors and partners to expand support to essential maternal and neonatal services at hospitals to cover the expected gap **as of Dec 1, 2025**.



Sexual Violence and Clinical Management of Rape Services

Globally, 1 in 3 women are subjected to physical or sexual violence by an intimate partner or sexual violence from a non-partner (WHO).

The Q3 2025 **Gender-Based Violence Information Management System (GBV IMS)** findings indicate that reported rape incidents constituted 3% of the total reported GBV cases, reflecting a consistent proportion with that recorded in Q2 2025 (3%). Although total reported GBV cases decreased by 4% in Q3 compared to Q2 2025, the percentage of reported rape incidents increased in Bekaa (4% in Q3 2025, compared to 3% in Q2 2025) and in Beirut/Mount Lebanon (5% in Q3 2025, compared to 4% in Q2 2025).

Sexual assault incidents decreased across all regions (10% in Q3 2025 compared to 14% in Q2 2025). However, the percentage of reported psychological/emotional abuse has significantly increased in Q3 2025, especially in the South. Partners attribute this to an increase in **verbal sexual assault** and abuse, which are reported under this category according to the global classification of GBV incidents.

The **MOPH PHENICS** reports **a 60% increase in beneficiaries of CMR services when compared to Q2 2025. This increase should not be interpreted solely as an rise in rape cases; it is likely also attributable to an increase in survivors reaching services and improved reporting by facilities.**

- 7/14 of MOPH CMR facilities have reported on cases for Q3 2025. The facilities are included in Nabatieh, Baalbak, Zahle, Beirut, and Tripoli districts.
- Key statistics include:
 - **Approx. 55% (43% in 2024) of reported survivors receiving CMR services at CMR clinics are children**
 - **Approx. 65% of reported survivors receiving CMR services at CMR clinics are female**
 - **Approx. 75% (57% in 2024) of reported survivors sought services at CMR facilities after > 72 hours of the rape incident.**

Challenges

- Delayed referral to CMR facility, beyond the critical 72 hours window indicated for essential time-sensitive medical and forensic services
- MOPH discontinued Daher El Bashq Gov. hospital due to mishandling a pediatric rape case in August
- Multiple barriers to timely access to forensic services (financial, fear of stigma, need to sensitize forensic doctors on CMR services...)
- Community awareness of CMR services remains low

In **Q3 2025**, health partners supported all **14 CMR facilities (8 hospitals and 6 PHCCs)** to offer **free-of-charge CMR services** to rape survivors of all nationalities.



This [video](#) raises awareness on sexual assault and provides critical information on CMR service provision

Updates From Partners

United Nations Population Fund (UNFPA)

Donors: KOICA, ECHO, SIDA, CERF, France, OCHA and UNFPA EF

UNFPA coordinated closely with the **MOPH**, the **Ministry of Social Affairs (MoSA)**, and local partners to adapt its response strategy to meet the emerging needs of the affected population and ensure continuity of essential SRH services at community, primary care and secondary care levels.

Service Delivery



UNFPA's support to **21 hospitals** across Lebanon, included financial coverage for childbirth costs which enhanced access to institutional based **safe delivery for 170 pregnant women**. UNFPA support included access to emergency obstetric care for pregnant women experiencing complications.



UNFPA's support to MOPH's network of **Clinical Management of Rape (CMR)** facilities through procurement and prepositioning of essential medical supplies and medication allows survivors of rape access to life saving medical services. **UNFPA** provided financial support for CMR services. This coverage initially included three CMR facilities at the start of Q3, and was later adjusted to two facilities after one was discontinued by the **Ministry of Public Health**.



In Q3, **UNFPA** supported **access to SRH services and information** including midwifery care and mental health support to a total of **18,370 beneficiaries** in partnership with SIDC, Akkarouna, Caritas, Salama, Amel, Makassed, SWSL, and Marsa in **31 PHCCs** and medical mobile units that operate in various communities across Lebanon. These services targeted vulnerable groups, including Syrian refugees, IDPs, people with disabilities, and the LGBTQI community. Support varied between **Comprehensive SRH services and Information**.

→ **SRH services and information**

Included pre-and postnatal care, menstrual management, sexually transmitted infections, family planning, and nutrition for pregnant and breastfeeding women etc UNFPA supported services includes medical consultation, midwifery care, subsidization of laboratory tests, sonography and RH tests, outreach awareness raising activities in addition to supporting referrals to specialized services as needed (i.e mental health care etc).

→ **Midwifery care and information on SRH**

In partnership with LOM, UNFPA deployed midwives to provide midwifery care and awareness on SRH at 10 PHCCs across Lebanon. reaching 1840 beneficiaries with midwifery care services and information



[UNFPA partners raise awareness on menstrual health](#)

Imam Sadr Foundation (ISF)

Donor: Korean International Cooperation Agency (KOICA) through UNFPA

Imam Sadr Foundation, supported by UNFPA, is offering ambulatory Sexual and Reproductive Health services through mobile teams or Primary Satellite Units, referring the service users to other complementary services when needed through its network of 6 PHCCs, community MHPSS, TBV, Rehabilitation center, school health, and/or palliative care free services. It offers Gyn/Obs. medical consultation, midwifery counseling and education, screening services, essential medicines, and healthy lifestyle modification.

"N. Houda, a 12 years old Syrian refugee, came with her mother to the SRH Community Clinic established through mobile health outreach in Tyre district. Her mother was so upset because she is not showing the expected signs of maturity, with an absence of the menstrual cycle, and repercussions on her mental well-being as she is bullied at school too.

ISF Gynecologist intervened by a clinical examination, and referral for an endocrinologist, with the relevant prescribed diagnostic tests. The results indicated a hereditary malformation, with the possibility of improvement following a certain therapy. The midwife and psychiatric nurse supported the family, and all together, with a person-centered approach, succeeded in understanding the situation and setting together a wellness adaptive plan. The girl and her mother were satisfied and happy to be able to change and to re-design a better future.



Community Health Worker providing awareness on SRH services at Imam Sader Foundation clinic in Tyre.

The Lebanese Association for Family Health - SALAMA

Donor: European Civil Protection and Humanitarian Aid Operations (ECHO) through UNFPA

Breaking the Silence — Empowering Women Through Integrated GBV and SRH Services

Mariam, a **32-year-old Syrian refugee** living in the Bekaa Valley, had endured years of emotional and physical violence from her husband. With **three young children and no income**, she felt trapped in silence and fear. The ongoing economic crisis and displacement had left her isolated, with no access to medical or psychological care.

One day, while attending a community awareness session organized by **SALAMA** under the **ECHO/UNFPA** project, Mariam learned about the availability of confidential GBV case workers and reproductive health services. Encouraged by the outreach worker's empathy and reassurance, she decided to visit SALAMA's clinic for the first time.

At the clinic, Mariam was warmly received by the GBV case worker, who provided a safe and confidential environment for her to share her experience. She was immediately linked to the midwife and gynecologist for a medical consultation after reporting symptoms of **pelvic pain and fatigue**. The medical team conducted a full **SRH assessment and provided treatment and counselling** on family planning options to help her regain control over her body and reproductive choices.

In parallel, the psychologist and caseworker offered emotional support and developed a tailored safety and recovery plan for her. Over the next few weeks, Mariam attended several psychosocial support sessions, where she began to rebuild her confidence and self-worth. The team also referred her for legal consultation regarding her protection options.

"I never imagined that I could feel safe again," Mariam said. "At SALAMA, I found care, respect, and a chance to start over."



Gynecologist performing ultrasound at SALAMA's El Karak Dispensary in Zahle

AMEL Association International

Donors: Agence Française de Développement (AFD), Federal Ministry for Economic Cooperation and Development (BMZ), European Civil Protection and Humanitarian Aid Operations (ECHO) through UNFPA

Promoting Early Detection and Health Equity: Awareness Campaigns Across Lebanon

During September and October, **Amel Association International** carried out a series of **breast health awareness campaigns** across its primary healthcare centers in Beqaa, Baalbek/Hermel, Mount Lebanon, South Lebanon, and Nabatieh. The initiative aimed to equip community members with accurate information on self-examination, early detection, and the benefits of timely screening for improved health outcomes.

The sessions fostered an open and supportive environment where participants could discuss concerns, ask questions, and access reliable guidance. As a direct result, around **500 women were referred for mammography screening**, demonstrating strong community engagement and increased trust in preventive health services.

These campaigns highlight **Amel's** continued commitment to advancing gender-responsive healthcare and ensuring equitable access to information, prevention, and early diagnosis for everyone.



Breast Health Awareness session delivered by AMEL's community outreach team at Bourj A Shamali in Tyre District.

SIDC – Advancing Rights-Based SRH and GBV Services Across Lebanon

Donors: European Civil Protection and Humanitarian Aid Operations (ECHO) through UNFPA, Care International, Expertise France...

Through its ongoing collaboration with **UNFPA**, **SIDC** is strengthening system resilience and ensuring the provision of integrated sexual and reproductive health (SRH) and gender-based violence (GBV) services at both primary and secondary healthcare levels. These initiatives guarantee that vulnerable communities have access to timely, high-quality, and inclusive SRHR services throughout the humanitarian–development continuum.

SIDC also implements complementary interventions with **CARE International**, **ACTED**, **IRC**, **Intersos**, **Trócaire**, and **Expertise France**, focusing on expanding access to essential SRH and protection services for women, girls, LGBTQI+ individuals, and other marginalized populations.

Key Achievements in 2025

➡ Learning & Empowerment:

SIDC trained over 30 media students in ethical and inclusive reporting, fostering a new generation of advocates who amplify vulnerable voices and challenge stigma.

➡ Building Bridges with ISF:

In continued collaboration with the Internal Security Forces (ISF), SIDC conducted an e-learning course on harm reduction, HIV, STIs, stigma & discrimination, SOPs, vulnerable populations... reinforcing trust between law enforcement and communities.

➡ Candlelight Memorial:

SIDC brought together communities, partners, and individuals affected by HIV/AIDS to honor lost lives, promote remembrance, and raise awareness on ending stigma and discrimination.

➡ Clinical Management of Rape:

The organization joined the Clinical Management of Rape (CMR) Taskforce in December 2024, ensuring functionality, accessibility, and coordination among CMR facilities nationwide, and facilitating timely referrals and follow-ups for survivors.

➡ Comprehensive Service Delivery:

Through its gynecology clinic in Beirut, SIDC provided ongoing gynecological consultations, mental health support, GBV case management, and emergency cash assistance to survivors of violence and individuals at risk. The clinic reached over 300 women from diverse nationalities including migrants and refugees, and is fully equipped to serve individuals with physical impairments, reflecting SIDC's commitment to accessibility and inclusion.

➡ Capacity Strengthening:

Frontline workers, educators, and human rights defenders benefited from targeted training on inclusion, protection, and psychosocial wellbeing.

➡ HIV Prevention and Care Expansion:

Under the "Gateway to Better Health" project led by Expertise France – L' initiative, SIDC extended HIV prevention and treatment services to South Lebanon and Akkar.

SIDC – Advancing Rights-Based SRH and GBV Services Across Lebanon

➔ SRH/GBV Integration:

Beyond service delivery, SIDC's outreach teams continue to identify and respond to complex protection and health cases. In one instance, the team supported a woman with a mental disability who had been subjected to exploitation and multiple pregnancies, activating the protection referral pathway to ensure safety, medical care, and psychosocial support for both the survivor and her newborn.

➔ Working with Marginalized Communities:

Across all interventions, SIDC integrates gender sensitivity, inclusion, and protection principles, ensuring that services remain accessible to vulnerable populations; including individuals with diverse SOGIESC, people who use drugs, sex workers, migrants, persons with disabilities and others. Through these collective efforts, SIDC continues to play a vital role in strengthening Lebanon's SRH and GBV response and upholding the right of every individual to health, dignity, and safety.



SIDC's outreach workers bringing SRH & GBV awareness directly to the communities in Bourj Hammoud

Médecins du Monde (MdM)

Donor: European Civil Protection and Humanitarian Aid Operations (ECHO)

Médecins du Monde (MdM) continued to advance the midwifery-led model of care at the Birth Center of Rafik Hariri University Hospital (RHUH) under the **WILADA project**. A notable increase in Normal Vaginal Deliveries at the Birth Center was recorded, rising from 32 in July and 32 in August, to 50 in September. This positive trend is significant in a university hospital context where an OBGYN-led model remains predominant, reflecting growing confidence in midwife-led and women-centered care.

Throughout Q3, **103 NVDs** were conducted at the Birth Center, with **96% led by midwives**, while residents/OB-GYNs intervened only as needed. Episiotomy rates remained **low (Q3 average: 9.6%)**, and per RHUH protocols, midwives do not perform suturing independently but manage most first- and second-degree tears with supervision or joint involvement as required.

MdM also supported safe care pathways, contributing to 267 timely referrals of high-risk pregnancies to the Delivery Suite or specialized services. These results highlight improved access, strengthened midwifery practice, and a gradual shift towards safe, evidence-based midwifery-led intrapartum care within a tertiary hospital setting.



MdM staff shares essential information with women attending the midwife/OBGYN clinic at Rafik Hariri University Hospital PHCC, ensuring they feel informed, supported, and engaged in their care

Premiere Urgence Internationale (PUI)

Donor: European Civil Protection and Humanitarian Aid Operations (ECHO)

Over 3 years (August 2022 to July 2025), with funding from ECHO, PUI ensured equitable access to quality maternal and emergency care for vulnerable Lebanese, Syrian, and displaced populations through agreements with hospitals across North, Akkar, South, Baalbeck, Nabatieh, Beirut, and Mount Lebanon. These partnerships supported the implementation of Birth Cash Assistance, Clinical Management of Rape, and Ambulatory Emergency Care services. Midwives and community health volunteers played a central role in identifying, referring, and following up on pregnant women, while task-shifting and mentoring within PHCCs strengthened service capacity and continuity. Even during the conflict escalation in 2024, remote counselling and strong coordination ensured uninterrupted care and safe referrals for displaced women.

Key Project Achievements



2932 Pregnant and Lactating Women accessed emergency obstetric and newborn care and post-abortion care



2471 eligible women were provided with transportation coverage



1824 patients benefitted from Ambulatory Emergency Care



2796 women received in-kind donations of mother and baby kits



75 rape survivors received covered forensic doctor fees



3,375 women provided with ANC consultations



7,023 women participated in counselling and education sessions



3,164 Pregnant Lactating Women referred from Primary Healthcare Centers to Secondary Healthcare Centers

The project was renewed by ECHO for 18 months (ends in Feb 2027) with the same scope of services.

Action Against Hunger (ACF)

Donor: The Swedish International Development Cooperation Agency (SIDA)

Supporting Women and Girls in Conflict-Affected Areas: ACF's MHM Campaign in South Lebanon

Access to safe and dignified menstrual hygiene management (MHM) remains a critical challenge for women and adolescent girls living in conflict-affected areas. Recognizing this need, Action Against Hunger (ACF) launched a targeted campaign to distribute MHM kits to vulnerable populations in Nabatiyeh and Bent Jbeil.

This initiative was implemented in close collaboration with local municipalities to ensure that distributions were both accessible and culturally appropriate. Each kit contained essential hygiene items designed to help women and girls manage their menstrual health safely and with dignity. In addition to distributing MHM kits, ACF also provided Multiple Micronutrient Supplements (MMS) to adolescents, aiming to improve their nutritional status and overall well-being.

Beyond the distribution, ACF prioritized health awareness and education. These sessions covered topics such as menstrual hygiene, reproductive health, nutrition, avoiding unhealthy habits, stress management, and general self-care practices. These sessions aimed to break taboos, promote knowledge, and empower women and girls to make informed decisions about their health and well-being.

One adolescent girl shared her thoughts after the session, saying:

"The session was very insightful and important for every young woman. It taught me a lot about the menstrual cycle, helped me understand my body better, and gave me the confidence not to fear or feel embarrassed about something so natural."

District	#Adolescent Girls Reached	#MHM Kits Distributed	Number of Adolescents receiving MMS
Nabatieh	284	258	136
Bent Jbeil	60	57	56
Total	344	315	192

To receive this newsletter, RHSWG announcements, and to join the working group [please sign up here](#).

The [RHSWG Google Drive Folder](#) contains repositories of SRH guidance documents, RHSWG and CMR TF MoM, communication materials, service mapping, and other useful documents.

All 35 organizations mentioned are active members of the RHSWG: ACF, Al Makassed, AMEL, BASD, Caritas Lebanon, Gawth, Humedica, IMC, IOCC Lebanon, IOM, IRC, ICRC, Lebanese Order of Midwives (LOM), La Chaîne De l'Espoir, LFPA, LRC, Mercy USA, MDM, MARSA, MEDAIR, MoPH, Makhzoumi, Order of Malta, Oxfam, Plan International, Proud Lebanon, PU-AMI, RI, SIDC, SALAMA, UNFPA, UNHCR, UNICEF, UNRWA, and WHO.

According to RHSWG SRH Service Mapping Exercise conducted in Q3 2025, the major donors supporting SRH interventions (by alphabetical order): AFD (France), BHA-USAID, BMZ-DHK (Germany), CERF, CDCS (The Crisis and Support Centre), EU-NDICI and ECHO (European Union), GAC (Canada), GFFO (Germany), Global Fund, IPPF, LHF, Monaco-Foundation Merieux, NORWAC, Private Funds, Republic of Korea, Swedish International Development Agency (SIDA)

For more information, please contact Houssein Sweid, RHSWG Coordinator @UNFPA: sweid@unfpa.org